



Board of Directors

Victor J. Reyes, MBA

Deloitte Consulting LLP
Chair

Shonta Chambers, MSW

Patient Advocate Foundation
Secretary

Brian Munroe

Bausch Health Companies, Inc.
Treasurer

Laurie Mobley

BRG Communications
Development Co-Chair

Nicholas Austin, JD

Microsoft Inc.

Gary Deverman, CFRE

NutriStyle

Ben Goodman

Maine Dept of Economic &
Community Development

Larry R. Holden

President & Chief Executive Officer

Melodie Narain-Blackwell

Color of Crohn's & Chronic Illness, Inc.

Global Liver Institute
100 M Street SE
Suite 750
Washington, DC 20003

✉ info@globalliver.org
🌐 globalliver.org

February 9, 2026

ASTP/ONC

Mary E. Switzer Building
Mailstop: 7033A
330 C Street S.W
Washington, DC 20201

Attn: HHS Health Sector AI RFI

To whom it may concern:

Global Liver Institute (GLI) appreciates the opportunity to provide concrete, experience-based feedback on the use of artificial intelligence (AI) in the health care sector.

GLI is a 501c(3) nonprofit organization founded in the belief that liver health must take its place on the global public health agenda commensurate with the prevalence and impact of liver illness. GLI promotes innovation, encourages collaboration, and supports the scaling of optimal approaches to help eradicate liver diseases. Operating globally, GLI is committed to solving the problems that matter to liver patients and equipping advocates to improve the lives of individuals and families impacted by liver disease and co-existing conditions such as obesity.

GLI agrees with the administration regarding the promise AI presents for health care. When used appropriately, AI has the potential to improve care delivery by supporting earlier disease

detection, reducing administrative burden, personalizing care, providing additional lifestyle guidance, and improving coordination across fragmented systems. For patients, this can mean faster diagnoses, clearer information, and more time with clinicians. For providers, AI can help streamline documentation and enhance decision support, allowing clinicians to focus more fully on direct patient care.

At the same time, AI has fallen short when deployed in high-stakes administrative functions, where its use has, in some cases, amplified delays, errors, and inequities rather than improving outcomes. In clinical decision-making, its use also increases patient stress, as patients report concerns about losing the human touch in their care and about the lack of transparency around when and how AI is used. The rapid pace of adoption underscores the need for clear guardrails to ensure these tools are used in ways that enhance clinical judgment, patient trust, and equitable access to care. Incorporating patient and caregiver experience into design and evaluation is essential to ensuring these tools improve outcomes and do not create new barriers.

AI Use in Coverage Decisions and Prior Authorization

One of the most consequential and concerning applications of AI in health care today is its use by payers in coverage determinations and prior authorization. Lawmakers across the political spectrum have acknowledged that traditional prior authorization processes already delay or deny necessary care. In a recent hearing held by the Energy and Commerce Committee, Committee Chair Brett Guthrie raised concerns about the impacts of AI, stating that “people are being denied care.” Committee Ranking Member Frank Pallone similarly warned that new AI-driven models risk creating “another barrier” for patients who already face significant obstacles to accessing care.

The use of AI to automate or accelerate coverage decisions raises the stakes further. When algorithms are trained on historical spending patterns rather than clinical need, they risk embedding past inequities into future decisions at scale. Lower spending is often misinterpreted as lower need, disproportionately affecting low-income, rural, and minority patients who have historically faced barriers to care. We have long held concerns about the influence of cost effectiveness analyses using quality-adjusted life years (QALYs) and similar measures that devalue people with disabilities and chronic conditions, underscoring the need for transparency of the data used to automate coverage decisions. Speed alone does not equal value if it comes at the cost of accuracy, fairness, or transparency.

Patient Harm, Bias, and Accountability Risks

The consequences of AI-driven denials are not hypothetical. National survey data show that 93 percent of physicians report prior authorization harms clinical outcomes, and nearly one in four has witnessed a serious adverse event — including hospitalization, disability, or death — linked to delayed or denied care.¹ Data showing that the majority of appealed denials are overturned suggests that many initial decisions are wrong, yet patients bear the burden of delay, confusion, and financial stress in the meantime.²

Bias in AI systems can go unnoticed until harm is already widespread. One widely cited example involved an algorithm used to identify patients for high-risk care management programs that significantly under-identified Black patients by relying on historical health care spending as a proxy for need.³ Black patients have historically received less care despite similar or greater illness burden, leading the algorithm to systematically exclude patients who would have benefited from additional support. When AI equates spending with need, it quietly carries inequities forward.

The Importance of Human Oversight and Transparency

AI tools should support, not replace, clinical judgment. When AI is used to inform clinical decision-making, clinicians should be transparent with patients about when and how these tools are assisting their care. Patients consistently express concern that increasing reliance on automated systems may erode the human connection that is central to effective, compassionate health care. Clear communication and clinician accountability are essential to maintaining trust and ensuring that AI augments — rather than substitutes for — meaningful patient-provider relationships.

Maintaining meaningful human oversight is particularly important in clinical settings, where AI-generated insights may influence diagnosis, treatment planning, or disease management. Over-reliance on automated recommendations risks undermining clinician expertise and could lead to inappropriate care if outputs are accepted without sufficient clinical context. AI systems should be designed to function as decision-support tools, with clinicians retaining full responsibility for interpreting and applying information based on individual patient needs.

Transparency must also extend to data governance. Patients should retain ownership of their personal health data, with clear protections governing how data are collected, used,

¹ <https://www.ama-assn.org/press-center/ama-press-releases/ama-survey-indicates-prior-authorization-wreaks-havoc-patient-care>

² <https://oig.hhs.gov/oei/reports/OEI-09-18-00260.pdf>

³ <https://www.science.org/doi/10.1126/science.aax2342>

shared, and stored when AI tools are deployed. Robust data protection policies are essential to preserving patient autonomy and preventing misuse of sensitive health information.

When considering potential policy changes, HHS should prioritize programmatic and payment policies that require meaningful human review and transparency for AI-enabled tools used in clinical care and administrative decision-making. This includes establishing standards for disclosure, auditability, data protections, and appeal rights when AI systems are used in Medicare and other federally-supported programs. Existing policies governing utilization management and clinical decision support should be revisited to ensure automated tools do not supplant clinical judgment or undermine beneficiary protections.

Building Patient Trust Through Governance and Participation

A core challenge in the current AI landscape is that patients — the individuals most directly affected by AI-enabled decisions — are rarely meaningfully involved in the design, governance, or evaluation of these tools. When patient voices and lived experience are absent, systems may prioritize efficiency or cost savings while overlooking real-world impacts on access, understanding, and trust.

HHS has an opportunity to set expectations that patient and caregiver input be integrated throughout the AI lifecycle, from development and testing to deployment and post-market evaluation. Transparency, shared decision-making, and patient-centered metrics are essential to building public confidence in AI-enabled care.

Aligning AI Innovation With Public Trust

HHS has stated its commitment to combating fraud, waste, and abuse while ensuring patients receive appropriate care. These goals are not mutually exclusive. However, AI models that financially benefit vendors or payers when care is withheld create misaligned incentives that warrant careful scrutiny. Clinicians have also called for greater investment in AI research so tools are rigorously tested for safety and effectiveness, deliver clear value, and do not introduce avoidable risks or costs. To ensure AI advances quality care rather than exacerbates disparities, HHS should pair stronger evidence generation with policies that align reimbursement and program design with patient outcomes, not short-term cost avoidance. AI can be a powerful tool for improving care, but only if accountability, transparency, and patient protections are built in from the start.

Conclusion

Thank you for the opportunity to comment on this RFI and for your consideration of these recommendations. HHS can accelerate responsible AI adoption by prioritizing transparency, meaningful human oversight in high-stakes decisions, and patient-centered evaluation before and after deployment. These guardrails will help ensure AI supports clinical care and strengthens public trust as these tools continue to scale.

Sincerely,



Larry R. Holden
President & Chief Executive Officer
Global Liver Institute

About Global Liver Institute

Global Liver Institute (GLI) is a 501(c)3 nonprofit organization founded in the belief that liver health must take its place on the global public health agenda commensurate with the prevalence and impact of liver illness. GLI promotes innovation, encourages collaboration, and supports the scaling of optimal approaches to help eradicate liver diseases. Operating globally, GLI is committed to solving the problems that matter to liver patients and equipping advocates to improve the lives of individuals and families impacted by liver disease. GLI holds Platinum Transparency with Candid/GuideStar, is a member of the National Health Council and NORD, and serves as a Healthy People 2030 Champion. Follow GLI on [Facebook](#), [Instagram](#), [LinkedIn](#), and [YouTube](#) or visit www.globalliver.org.

